

Stream — Field One-Pagers

Pediatrics · Direct Primary Care · Internal Medicine 19 Aug 2026 For the GTM team — not for distribution to prospects

SEGMENT 01

Independent Pediatrics

The deepest thing we've built and the least proven. Sell the fax pile today; sell the well-visit panel as what it grows into.

WHO TO CALL, WHO TO SKIP

CALL

- + **3–15 clinicians** — three is the floor, four or more is the target
- + Owner-physician or practice manager — they own both the schedule and the money
- + No EHR, a cheap one, or one whose recall report nobody trusts
- + Already bought a scribe (they've cleared the AI objection and found it didn't fix the paperwork)

SKIP

- Hospital-owned or system-affiliated
- 25+ clinicians, or anyone with an IT director
- **Under 3 clinicians** — self-serve trial, not a booked demo
- Anyone who makes EHR write-back a condition of the trial

THE OPENER

When an ED discharge summary comes back for one of your patients, who reads it — and where does it end up? Every practice has a bad answer: scanned to a folder, or sitting in a pile. You're asking about a workflow, not claiming a capability — that's why it gets replies. Subject lines to test: *the discharge summary pile · who reads your faxes*. Never "AI scribe," never "save 2 hours a day."

THE THREE BEATS

- 1 Walk in ready.** Before you open the door: every active problem, everything the outside world sent, and what's waiting on you — one screen.
- 2 Leave with the note done.** Records the visit, writes it up problem by problem in your voice, suggests the codes. One click to paste wherever it goes.
- 3 Nothing falls off.** Faxes and discharge summaries read themselves and file under the right problem. Well visits surface on the AAP schedule as your panel comes through.

THE DEMO — 20 SECONDS, RUN IT ON EVERY CALL

Upload a real ED discharge summary. Watch Stream pull out the clinical content and file it under the right problems. Click **View source** on the surprising one and

land on the highlighted sentence in the original fax. Then stop talking.

Second demo, *only* if `task_rules` is on for that account: filter the panel to 1–5y, sort by days overdue, export CSV. "That's your call list."

THE NUMBER — MAKE THEM BUILD IT

Named references yes, time-saved statistic no — don't invent one. Ask: *what do you collect on a well visit, and how many kids are behind?* Multiply it in front of them; their number beats ours.

Say this before they discover it.

The engine only counts visits it has seen, and won't accuse a family of missing one that happened before Stream arrived. The panel fills in as patients come through — useful in months, complete after a full visit cycle. **The document workflow works the first week.** Lead with that and let the panel compound; never promise a populated call list on day one.

OBJECTIONS

"We already have an EHR."

Good — keep it. Stream works alongside it. Nothing to install, no interface, no IT project, no implementation fee.

"Our EHR already does recall."

Pull it up while we're on the call. Does it know the difference between a visit that's clinically due and one the payer will cover yet? And does it reach the clinician at the visit, or land in a report nobody opens?

"So we type everything twice?"

One click, formatted for any field — rich text and plain text at once, or section by section. It's a paste, not a retype. *Two sentences, then move on; arguing a fair objection only makes it bigger.*

"Does it make things up?"

Every claim links back to the exact sentence it came from, and anything unverifiable is suppressed. Outside records never silently become the clinician's words. Show it — don't describe it.

PRICING

Pro \$149/clinician/mo, or **\$99 annually** · staff seats **\$25** (first free) · 30-day trial, no card · uploads included · fax number **free for 30 days**.

SEGMENT 02

Direct Primary Care & Concierge

The only segment where we aren't an add-on to something else, and the shortest sales cycle we have. But most DPC practices are solo or two — the qualifying pool here is small, so work it opportunistically behind pediatrics and internal medicine.

WHO TO CALL, WHO TO SKIP

CALL

+ DPC, concierge or cash-pay groups, **3+ clinicians** (four or more preferred)

+ The founding physician is the buyer *and* a user — no committee, decision in one call

+ Strongest signal: no EHR, or one they complain about by name

+ Newly forming groups at 3+ — zero switching cost, actively shopping

SKIP

– Anyone who needs us to e-prescribe

– Anyone who needs claims or a superbill from us

– Anyone who needs a patient portal

– **Solo and two-clinician practices** — the classic DPC shape, and below our booking floor. Self-serve trial only

THE OPENER

You left to spend more time with patients. Did the documentation follow you out?

These physicians left fee-for-service for longer visits and smaller panels. Chart work is the part that came with them — and most are running either nothing or something cheap they resent paying for.

THE THREE BEATS

1 It's the chart, not a bolt-on. Patients, problems, visit history, documents — one place. No EHR required, and nothing to integrate.

2 The visit writes itself. Record from your phone, your laptop, or a telehealth tab. The note comes back problem by problem, in your voice, and it learns your style.

3 The outside world files itself. Labs, consults and hospital notes come back and land under the right problem — each one linked to the sentence it came from.

BRING THESE UP WHEN THEY FIT — DON'T RECITE THEM

▪ Plain-language visit summaries to hand a member. For a membership practice that's a retention artifact, not a clinical one.

- Referral letters and prior auths generated as faxable PDFs, with your signature block.
- Lab requisitions and sample tracking.
- A weekly reading list built from what you actually saw last week. DPC is professionally isolating; this lands harder here than anywhere else.

THE DEMO

Skip the note — every scribe demos the note. Open a chart and show the problem list *assembled from their outside records*, then upload a specialist letter and watch it attach itself. For a doctor who keeps that list by hand in a Word document, that's the moment.

Name the boundary on call one, not call three.

Stream is the clinical chart and the documentation. It does **not** e-prescribe, file claims, or provide a patient portal. Ask what they use for those and confirm they're keeping them. A physician who finds out in week two churns and tells every other DPC physician they know; one who hears it in minute five trusts everything else you said.

OBJECTIONS

"Is this an EHR?"

No, and we're not trying to be. It's your chart and your documentation. Prescribing and billing stay where they are.

"I heard notes get deleted after 48 hours."

That's Lite — a standalone scribe with no chart. Pro keeps the chart. *Reps: never put a DPC practice on Lite.*

"I'm solo. Isn't this overkill?"

The trial's free and you're set up the same afternoon. There's no implementation to be overkill about.

"What does it cost me against my EHR?"

Compare, don't discount. \$149/month against what they're paying for software they've just finished complaining about.

PRICING

Pro \$149/clinician/month, or \$99 billed annually · 30-day trial, no card · USD and CAD. Document upload included; provisioned fax number free for the first 30 days.

SEGMENT 03

Internal Medicine & Risk-Bearing Primary Care

Heaviest fax volume of the three, and the only segment where the buyer already has a dollar figure in their head. Qualify hard on risk exposure.

WHO TO CALL, WHO TO SKIP

CALL

- + Independent IM / adult primary care, **3–20 clinicians** (four or more preferred)
- + Risk exposure: Medicare Advantage, ACO participation, any capitated contract
- + Buyer is the owner, managing partner, or whoever answers for risk and quality

SKIP

- Health-system owned
- **Under 3 clinicians** — self-serve trial, not a booked demo
- Pure fee-for-service with no fax pain — that's a scribe buyer who will grind you on price

THE OPENER

Which of your Medicare Advantage patients haven't had their conditions documented this year — and how do you find out?

The answer is a spreadsheet, a vendor report that arrives two months late, or nobody. If they're not risk-bearing, fall back to: *"Who reads the hospital discharge summaries?"*

THE THREE BEATS

- 1 Walk in ready.** Four tabs before you open the door: what's active, what's falling off the chart, what's HCC-coded and not yet documented this year, and what's waiting on your review.
- 2 An eight-problem patient gets an eight-problem note.** Each condition documented separately with its own assessment and plan — not one paragraph somebody has to unpick later. Codes suggested, then sharpened to the more specific one.
- 3 Nothing falls off.** Discharge summaries and consult letters file under the right problem. A condition that shows up *only* in the outside record becomes a real problem on the chart, flagged external — the hospital found something your chart didn't know about.

THE DEMO

Discharge summary first, same as every segment. Then open the pre-visit view and show **Falling Off** — problems

last documented ten or eleven months ago, red badge at eleven. Say: *"That's your December scramble, in August."*

THE NUMBER — MAKE THEM BUILD IT

Ask what their average RAF is and what a recaptured condition is worth under their contract. Stream carries HCC weights to four decimal places and shows them on the problem itself. Their finance person already owns the multiplier; you just supply the conditions they're about to lose.

Honest ramp.

The HCC surfaces work from what's documented in Stream, so they sharpen over the first months as patients cycle through — faster here than pediatrics, because these patients come in three or four times a year. The document workflow works week one. Say both.

OBJECTIONS

"Our ACO already sends us gap reports."

Those are retrospective and claims-based — they tell you what you missed. This tells the clinician what's open while the patient is still in the room. Different job, different month.

"That's double documentation."

It is a paste, not a retype — one click, formatted for any field, section by section if that's how your EHR wants it. Acknowledge and move.

"Does it hallucinate?"

Every outside claim links to the highlighted sentence it came from; anything unverifiable is suppressed; outside content never enters a note without the clinician putting it there. Demo it.

"What about compliance?"

BAA included in our terms — no separate negotiation. Per-tenant database isolation, role-based access control, audit logging, two-factor authentication. **Do not say SOC 2.**

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APPLIES TO EVERY SEGMENT

Guardrails

Everything here was verified against the product, not the roadmap. A claim from the left column is the fastest way to lose a deal in diligence.

Booking floor — 3 clinicians minimum, 4 or more preferred.

No demo with a practice under three clinicians. At \$149 a seat, a one- or two-person practice doesn't cover the cost of sourcing the meeting — that demand is real, but it arrives through the self-serve trial, not a booked call.

Confirm headcount before the invite goes out. Count physicians, NPs and PAs; not front-desk, billing or MAs — those are \$25 seats.

NEVER SAY / SAY INSTEAD

NEVER SAY	SAY INSTEAD
✗ Integrates with your EHR · syncs with · connects to · writes back	Works alongside any EHR. No integration, no IT project, no implementation fee. <i>(The old "native integrations coming" claim was removed from the website in Aug 2026 — it was never true. Don't reintroduce it on a call.)</i>
✗ SOC 2 compliant / certified	BAA included in our terms. Per-tenant data isolation, role-based access control, audit logging, two-factor authentication.
✗ HIPAA certified	HIPAA-aligned architecture. (No vendor can be HIPAA certified — the phrase reads as amateur to anyone who'd know.)
✗ Your overdue list is ready on day one	The document workflow works week one. The panel view fills in as your patients come through.
✗ Any mention of Form Filler, automatic chart extraction into structured tables, or style-learning from a sample note	Nothing. Not shipped. Don't hint at it either.
✗ Unlimited history on every plan	Lite / Quick Encounter deletes after 48 hours — state it plainly in all Lite material or it's a refund and a support fire. Lite (\$59, scribe only, no chart) isn't publicly sold: never put a practice on it.
✗ Clinicians save X minutes a day	We have no time-saved statistic — don't invent one. We <i>do</i> have three named physician references and 30,000+ documented encounters. Use those, and build the ROI number out of the prospect's own figures.

BEFORE ANY DEMO

Confirm flag state on the account — these are per-tenant and may be off:

task_rules document_excerpts tasks_module

outbound_faxes journal_club help_chatbot

merge_inline_recommendation

Documents still carries a **Beta** badge. One-click demo charts exist for family medicine, pediatrics, cardiology, endocrinology and behavioral health — ask Jacob for access, and **demo a pediatrician a pediatric panel.**

SET THESE EXPECTATIONS IN ONBOARDING, NOT IN THE WILD

- Fax needs a Spruce account or a Stream-provisioned number: ~15 minutes of admin *plus* a manual step on our side. Not self-serve — don't promise same-day fax.

- Excerpt review is one-way: Accept and No-action are final (Re-tag is the exception).
- Regenerating a note replaces manual edits. Teach generate-once-then-edit-in-place — the most common way a new user loses work.

WHAT WE STILL OWE THE FIELD

- **Proof.** Three named physician references and 30,000+ documented encounters exist and are on the website — use them. Still missing: retention figures and a defensible time-saved number, so ROI is still built from the prospect's own numbers.
- **A security page.** Small-practice buyers ask, and the Trust Center link is currently commented out of the app.
- **Prior-well-visit import.** The single build that would turn the pediatrics sheet from a promise into a demo.

Derived from the engineering handoff of 19 Aug 2026 and a direct read of the source. Claims about what the product does were checked in code; claims about who wants it are hypotheses with one design partner behind them. Update this sheet when that changes.